SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

Nivolumab and ipilimumab for advanced melanoma

NCCP 00431a: advanced unresectable or metastatic melanoma in adults. This patient guide is the melanoma context, not the separate liver-cancer indication. · NCCP 00431 · version 11 · 21-day cycle

Patient name	DOB	Hospital no.	Date

Treatment context
Nivolumab and ipilimumab help immune cells recognise and attack melanoma. The combination phase is followed by nivolumab alone when appropriate. The aim in this setting is to control advanced disease; benefit and toxicity are reviewed for the individual.

Schedule
The combination phase gives nivolumab followed by ipilimumab by IV infusion every three weeks for up to four doses. If appropriate, nivolumab alone then continues on the team-selected two-weekly or four-weekly schedule, by the prescribed IV or subcutaneous route. After the last combination dose, the first two-weekly maintenance dose starts three weeks later; the first four-weekly maintenance dose starts six weeks later in this melanoma schedule. Treatment continues until progression or unacceptable toxicity, subject to review.

1. TREATMENT DISCUSSION

- ☐ Purpose, expected benefit and alternatives explained
- ☐ Indication, medicines, sequence and duration confirmed
- ☐ Blood tests and any individually indicated heart checks discussed
- ☐ Support medicines and phase changes explained
- ☐ Fertility, contraception and breastfeeding discussed where relevant
- ☐ Common, serious and lasting drug-specific risks discussed below
- ☐ Urgent symptoms and local contact details confirmed

2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

Nivolumab
PD-1 CHECKPOINT IMMUNOTHERAPY

- ☐ Immune inflammation of bowel, lungs, liver or kidneys
- ☐ Thyroid, pituitary, adrenal and glucose problems
- ☐ Skin reactions and rare severe skin injury
- ☐ Rare heart, muscle, neurological and eye inflammation
- ☐ Infusion or injection reactions
- ☐ Some immune effects need long-term treatment

Ipilimumab
CTLA-4 CHECKPOINT IMMUNOTHERAPY

- ☐ Immune colitis and severe diarrhoea
- ☐ Liver, endocrine and skin inflammation
- ☐ Rare neurological, eye, heart or muscle inflammation
- ☐ Infusion reactions
- ☐ Immune effects during or after treatment

Discuss the exact diagnosis, treatment intent, expected benefits and uncertainty, suitable alternatives, the regimen-specific risks and monitoring, fertility and pregnancy precautions, and practical burdens. Confirm the treatment phase and route. This guide supports the conversation and does not replace individual consent.

Individual aim / benefit, alternatives, material risks and questions:

3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

Prototype only. This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00431 v11.

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